

PACKAGE INSERT

1. NAME OF THE MEDICINAL PRODUCT

Letimble 10 mg Tablets.

2. QUALITATIVE AND QUANTITATIVE COMPOSITION

Each tablet contains 10 mg of ezetimibe.

Excipient(s) with known effect: 67.50 mg lactose monohydrate.

For the full list of excipients, see section 6.1.

3. PHARMACEUTICAL FORM

Tablet.

White to almost white, oval tablets with debossing "10" on one side and "EZT" on the other side.

4. CLINICAL PARTICULARS

4.1 Therapeutic indications

Primary hypercholesterolaemia

Ezetimibe, administered alone, or with an HMG-CoA reductase inhibitor (statin), is indicated as adjunctive therapy to diet in patients with primary (heterozygous familial and non-familial) hypercholesterolaemia.

Ezetimibe, administered in combination with fenofibrate, is indicated as adjunctive therapy to diet for the reduction of elevated total-C, LDL-C, Apo B and non-HDL-C in patients with mixed hyperlipidemia.

Prevention of Cardiovascular Events

Ezetimibe, administered with a statin, is indicated to reduce the risk of cardiovascular events (cardiovascular death, nonfatal myocardial infarction, nonfatal stroke, hospitalization for unstable angina, or need for revascularization), in patients with coronary heart disease (CHD) and a history of acute coronary syndrome (ACS).

Homozygous Familial Hypercholesterolaemia (HoFH)

Ezetimibe, administered with a statin, is indicated for patients with HoFH. Patients may also receive adjunctive treatments (e.g. LDL apheresis).

Homozygous Sitosterolaemia (Phytosterolaemia)

Ezetimibe is indicated for the reduction of elevated sitosterol and campesterol levels in patients with homozygous familial sitosterolaemia.

4.2 Posology and method of administration

Posology

The patient should be on an appropriate lipid-lowering diet and should continue on this diet during treatment with ezetimibe.

Use in Patients with Primary Hypercholesterolaemia

The recommended dose of ezetimibe is 10 mg once daily, used alone, with a statin or with fenofibrate. Ezetimibe can be administered at any time of the day, with or without food.

Ezetimibe may be administered with a statin (in patients with primary hypercholesterolemia) or with fenofibrate (in patients with mixed hyperlipidemia) for incremental effect. For convenience, the daily dose of Ezetimibe may be taken at the same time as the statin or fenofibrate, according to the dosing recommendations for the respective medications.

If ezetimibe 10mg tablets are used in combination with a statin therapy, the dose instructions for that particular statin should be consulted.

When initiating lipid lowering treatment, which includes ezetimibe 10mg tablets and a statin in combination, the indicated usual initial dose of that particular statin should be used or the already established higher statin dose should be continued.

If the statin dose is to be increased for the first time or further, the dose instructions of that particular statin should be followed (such as dose increase only after at least 4 weeks of regular use of the combination without any change).

The stepwise increase of the statin dose in combination treatment results in a relatively small additional decrease of LDL-C, but increases the risk of dose-related adverse events of the statin. This has to be considered for the risk-benefit-assessment when the statin dose is considered.

Use in Patients with Coronary Heart Disease Combination Therapy with a Statin

For incremental cardiovascular event reduction in patients with coronary heart disease, ezetimibe 10 mg may be administered with a statin with proven cardiovascular benefit.

Use in the Elderly

No dose adjustment is required for elderly patients. However, greater sensitivity of some older individuals cannot be ruled out.

Use in Pediatric Patients

Children and adolescents ≥ 10 years: No dose adjustment is required.

Children < 10 years: Treatment with ezetimibe is not recommended.

Use in Hepatic Impairment

No dose adjustment is required in patients with mild hepatic insufficiency (Child Pugh score 5 to 6). Treatment with ezetimibe is not recommended in patients with moderate (Child Pugh score 7 to 9) or severe (Child Pugh score > 9) liver dysfunction.

Use in Renal Impairment

No dose adjustment is required for renally impaired patients.

Co-administration with bile acid sequestrants

Dosing of ezetimibe should occur either ≥ 2 hours before or ≥ 4 hours after administration of a bile acid sequestrant.

Method of administration

For oral use.

4.3 Contraindications

Hypersensitivity to the active substance or to any of the excipients.

When ezetimibe is co-administered with a statin, please refer to the SmPC for that particular medicinal product.

Therapy with ezetimibe co-administered with a statin is contraindicated during pregnancy and lactation.

Ezetimibe co-administered with a statin is contraindicated in patients with active liver disease or unexpected persistent elevations in serum transaminases.

4.4 Special warnings and precautions for use

When ezetimibe is co-administered with a statin, please refer to the SmPC for that particular medicinal product.

Liver enzymes

In patients receiving ezetimibe with a statin, consecutive transaminase elevations ($\geq 3 \times$ the upper limit of normal [ULN]) have been observed. When ezetimibe is co-administered with a statin, liver function tests should be performed at initiation of therapy and according to the recommendations of the statin (see section 4.8).

Skeletal muscle

In post-marketing experience with ezetimibe, cases of myopathy and rhabdomyolysis have been reported. Most patients who developed rhabdomyolysis were taking a statin concomitantly with ezetimibe. However, rhabdomyolysis has been reported very rarely with ezetimibe monotherapy and very rarely with the addition of ezetimibe to other agents known to be associated with increased risk of rhabdomyolysis. If myopathy is suspected based on muscle symptoms or is confirmed by a creatine phosphokinase (CPK) level > 10 times the ULN, ezetimibe, any statin, and any of these other agents that the patient is taking concomitantly should be immediately discontinued. All patients starting therapy with ezetimibe should be advised of the risk of myopathy and told to report promptly any unexplained muscle pain, tenderness or weakness (see section 4.8).

Patients with hepatic impairment

Due to the unknown effects of the increased exposure to ezetimibe in patients with moderate or severe hepatic impairment, ezetimibe is not recommended (see section 5.2).

Fibrates

The safety and efficacy of ezetimibe administered with fibrates have not been established.

If cholelithiasis is suspected in a patient receiving ezetimibe and fenofibrate, gallbladder investigations are indicated and this therapy should be discontinued (see sections 4.5 and 4.8).

Cyclosporine

Caution should be exercised when initiating ezetimibe in the setting of cyclosporine. Cyclosporine concentrations should be monitored in patients receiving ezetimibe and cyclosporine (see section 4.5).

Anticoagulants

If ezetimibe is added to warfarin, another coumarin anticoagulant, or fluindione, the International Normalised Ratio (INR) should be appropriately monitored (see section 4.5).

4.5 Interaction with other medicinal products and other forms of interaction

It has been shown that ezetimibe does not induce cytochrome P450 medicinal products metabolising enzymes. No clinically significant pharmacokinetic interactions have been observed between ezetimibe and medicinal products known to be metabolised by cytochromes P450 1A2, 2D6, 2C8, 2C9, and 3A4, or N-acetyltransferase.

Ezetimibe had no effect on the pharmacokinetics of dapsone, dextromethorphan, digoxin, oral contraceptives (ethinyl estradiol and levonorgestrel), glipizide, tolbutamide, or midazolam, during co-administration. Cimetidine, co-administered with ezetimibe, had no effect on the bioavailability of ezetimibe.

Antacids

Concomitant antacid administration decreased the rate of absorption of ezetimibe but had no effect on the bioavailability of ezetimibe. This decreased rate of absorption is not considered clinically significant.

Cholestyramine

Concomitant cholestyramine administration decreased the mean area under the curve (AUC) of total ezetimibe (ezetimibe + ezetimibe glucuronide) approximately 55%. The incremental low-density lipoprotein cholesterol (LDL-C) reduction due to adding ezetimibe to cholestyramine may be lessened by this interaction.

Fibrates

In patients receiving fenofibrate and ezetimibe, physicians should be aware of the possible risk of cholelithiasis and gallbladder disease (see section 4.4 and 4.8).

If cholelithiasis is suspected in a patient receiving ezetimibe and fenofibrate, gallbladder investigations are indicated and this therapy should be discontinued (see section 4.8).

Concomitant fenofibrate or gemfibrozil administration modestly increased total ezetimibe concentrations (approximately 1.5- and 1.7-fold respectively).

Co-administration of ezetimibe with other fibrates has not been studied.

Fibrates may increase cholesterol excretion into the bile, leading to cholelithiasis. In animal studies, ezetimibe sometimes increased cholesterol in the gallbladder bile, but not in all species. A lithogenic risk associated with the therapeutic use of ezetimibe cannot be ruled out.

Statins

No clinically significant pharmacokinetic interactions were seen when ezetimibe was co-administered with atorvastatin, simvastatin, pravastatin, lovastatin, fluvastatin or rosuvastatin.

Cyclosporine

Caution should be exercised when initiating ezetimibe in the setting of cyclosporine. Cyclosporine concentrations should be monitored in patients receiving ezetimibe and cyclosporine (see section 4.4).

Anticoagulants

Concomitant administration of ezetimibe (10 mg once daily) had no significant effect on bioavailability of warfarin and prothrombin time. However, there have been reports of increased International Normalised Ratio (INR) in patients who had ezetimibe added to warfarin or fluindione. If ezetimibe is added to warfarin, another coumarin anticoagulant, or fluindione, INR should be appropriately monitored (see section 4.4).

Paediatric population

Interaction studies have only been performed in adults.

4.6 Pregnancy and lactation

Ezetimibe co-administered with a statin is contraindicated during pregnancy and lactation (see section 4.3), please refer to the package insert for that particular statin.

Pregnancy

Ezetimibe should be given to pregnant women only if clearly necessary. No clinical data are available on the use of ezetimibe during pregnancy. The use of ezetimibe in monotherapy have shown no evidence of direct or

indirect harmful effects on pregnancy, embryofoetal development, birth or postnatal development.

Lactation

Ezetimibe should not be used during lactation. Ezetimibe has been shown to be secreted into breast milk in animals. It is not known if ezetimibe is secreted into human breast milk.

Fertility

No clinical trial data are available on the effects of ezetimibe on human fertility. Ezetimibe had no effect on the fertility of male or female rats.

4.7 Effects on ability to drive and use machines

No studies on the effects on the ability to drive and use machines have been performed. However, when driving vehicles or operating machines, it should be taken into account that dizziness has been reported.

4.8 Undesirable effects

Adverse reactions were usually mild and transient. The overall incidence of side effects was similar between ezetimibe and placebo. Similarly, the discontinuation rate due to adverse experiences was comparable between ezetimibe and placebo.

The following adverse reactions were observed in patients treated with ezetimibe and at a greater incidence than placebo or in patients treated with ezetimibe co-administered with a statin and at a greater incidence than statin administered alone.

Post-marketing adverse reactions were derived from reports containing ezetimibe either administered alone or with a statin.

Ezetimibe monotherapy

Metabolism and nutrition disorders

Uncommon: Decreased appetite

Vascular disorders

Uncommon: Hot flush, hypertension

Respiratory, thoracic and mediastinal disorders

Uncommon: Cough

Gastrointestinal disorders

Common: Abdominal pain, diarrhoea, flatulence

Uncommon: Dyspepsia, gastrooesophageal reflux disease, nausea

Musculoskeletal and connective tissue disorders

Uncommon: Arthralgia, muscle spasms, neck pain

General disorders and administration site condition

Common: Fatigue

Uncommon: Chest pain, pain

Investigations

Uncommon: ALT and/or AST increased, blood CPK increased, gamma-glutamyltransferase increased, liver function test abnormal

Additional adverse reactions with ezetimibe co-administered with a statin

Nervous system disorders

Common: Headache

Uncommon: Paraesthesia

Gastrointestinal disorders

Uncommon: Dry mouth, gastritis

Skin and subcutaneous tissue disorders

Uncommon: Pruritus, rash, urticaria

Musculoskeletal and connective tissue disorders

Common: Myalgia

Uncommon: Back pain, muscular weakness, pain in extremity

General disorders and administration site condition

Uncommon: Asthenia, oedema peripheral

Investigations

Common: ALT and/or AST increased

Post-marketing experience (with or without a statin)

Blood and lymphatic system disorders

Not known: Thrombocytopenia

Immune system disorders

Not known: Hypersensitivity, including rash, urticaria, anaphylaxis and angioedema

Psychiatric disorders

Not known: Depression

Nervous system disorders

Not known: Dizziness; paraesthesia

Respiratory, thoracic and mediastinal disorders

Not known: Dyspnoea

Gastrointestinal disorders

Not known: Pancreatitis, constipation

Hepatobiliary disorders

Not known: Hepatitis, cholelithiasis, cholecystitis

Skin and subcutaneous tissue disorders

Not known: Erythema multiforme

Musculoskeletal and connective tissue disorders

Not known: Myalgia; myopathy/rhabdomyolysis (see section 4.4)

General disorders and administration site conditions

Not known: Asthenia

Ezetimibe co-administered with fenofibrate

Gastrointestinal disorders

Common: Abdominal pain

4.9 Overdose

A few cases of overdose with ezetimibe have been reported: most have not been associated with adverse experiences. Reported adverse experiences have not been serious. In the event of an overdose, symptomatic and supportive measures should be employed.

5. PHARMACOLOGICAL PROPERTIES**5.1 Pharmacodynamic properties**

Pharmacotherapeutic group: Lipid modifying agents, plain, other lipid modifying agents.

ATC code: C10AX09.

Mechanism of action

Ezetimibe is in a new class of lipid-lowering compounds that selectively inhibit the intestinal absorption of cholesterol and related plant sterols. Ezetimibe is orally active, and has a mechanism of action that differs from other classes of cholesterol-reducing compounds (e.g. statins, bile acid sequestrants [resins], fibric acid derivatives, and plant stanols). The molecular target of ezetimibe is the sterol transporter, Niemann-Pick C1-Like 1 (NPC1L1), which is responsible for the intestinal uptake of cholesterol and phytosterols.

Ezetimibe localises at the brush border of the small intestine and inhibits the absorption of cholesterol, leading to a decrease in the delivery of intestinal cholesterol to the liver; statins reduce cholesterol synthesis in the liver and together these distinct mechanisms provide complementary cholesterol reduction.

Ezetimibe inhibits the absorption of ¹⁴C-cholesterol with no effect on the absorption of triglycerides, fatty acids, bile acids, progesterone, ethinyl estradiol, or fat soluble vitamins A and D.

Cardiovascular morbidity and mortality vary directly with the level of total-C and LDL-C and inversely with the

level of HDL-C.

Administration of ezetimibe with a statin is effective in reducing the risk of cardiovascular events in patients with coronary heart disease and ACS event history.

5.2 Pharmacokinetic properties

Absorption

After oral administration, ezetimibe is rapidly absorbed and extensively conjugated to a pharmacologically active phenolic glucuronide (ezetimibe-glucuronide). Mean maximum plasma concentrations (C_{max}) occur within 1 to 2 hours for ezetimibe-glucuronide and 4 to 12 hours for ezetimibe. The absolute bioavailability of ezetimibe cannot be determined as the compound is virtually insoluble in aqueous media suitable for injection.

Concomitant food administration (high fat or non-fat meals) had no effect on the oral bioavailability of ezetimibe when administered as Letimble 10 mg tablets. Ezetimibe can be administered with or without food.

Distribution

Ezetimibe and ezetimibe-glucuronide are bound 99.7% and 88 to 92% to human plasma proteins, respectively.

Biotransformation

Ezetimibe is metabolised primarily in the small intestine and liver via glucuronide conjugation (a phase II reaction) with subsequent biliary excretion. Minimal oxidative metabolism (a phase I reaction) has been observed in all species evaluated. Ezetimibe and ezetimibe-glucuronide are the major drug-derived compounds detected in plasma, constituting approximately 10 to 20% and 80 to 90% of the total active substance in plasma, respectively. Both ezetimibe and ezetimibe-glucuronide are slowly eliminated from plasma with evidence of significant enterohepatic recycling.

The half-life for ezetimibe and ezetimibe-glucuronide is approximately 22 hours.

Elimination

Following oral administration of ^{14}C -ezetimibe (20 mg) to human subjects, total ezetimibe accounted for approximately 93% of the total radioactivity in plasma. Approximately 78% and 11% of the administered radioactivity were recovered in the faeces and urine, respectively, over a 10-day collection period. After 48 hours, there were no detectable levels of radioactivity in the plasma.

Special Populations

Paediatric population

The pharmacokinetics of ezetimibe are similar between children ≥ 6 years and adults. Pharmacokinetic data in the paediatric population < 6 years of age are not available. Clinical experience in paediatric and adolescent patients includes patients with HoFH, HeFH, or sitosterolaemia.

Older people

Plasma concentrations for total ezetimibe are about 2-fold higher in the elderly (≥ 65 years) than in the young (18 to 45 years). LDL-C reduction and safety profile are comparable between elderly and young subjects treated with ezetimibe. Therefore, no dose adjustment is necessary in the elderly.

Hepatic impairment

After a single 10mg dose of ezetimibe, the mean AUC for total ezetimibe was increased approximately 1.7-fold in patients with mild hepatic impairment (Child-Pugh score 5 or 6). In patients with moderate hepatic impairment (Child-Pugh score 7 to 9), the mean AUC for total ezetimibe was increased approximately 4-fold. No dose adjustment is necessary for patients with mild hepatic impairment. Due to the unknown effects of the increased exposure to ezetimibe in patients with moderate or severe (Child-Pugh score > 9) hepatic impairment, ezetimibe is not recommended in these patients (see section 4.4).

Renal impairment

After a single 10mg dose of ezetimibe in patients with severe renal disease, the mean AUC for total ezetimibe was increased approximately 1.5-fold. However, no dose adjustment is necessary for renally impaired patients.

Gender

Plasma concentrations for total ezetimibe are slightly higher (approximately 20%) in women than in men. LDL-C reduction and safety profile are comparable between men and women treated with ezetimibe. Therefore, no dose adjustment is necessary on the basis of gender.

6. PHARMACEUTICAL PARTICULARS

6.1 List of excipients

Lactose monohydrate
Hypromellose

Croscarmellose sodium
Microcrystalline cellulose
Sodium lauryl sulfate
Magnesium stearate

6.2 Incompatibilities

Not applicable.

6.3 Shelf life

3 years.

6.4 Special precautions for storage

Store below 30°C, in the original package in order to protect from moisture.
Keep out of the reach and sight of children.

6.5 Nature and contents of container

Ezetimibe 10 mg tablets are packed into Al/Al blister.
Pack sizes available are 30, 60, and 100 tablets.

Not all registered pack sizes may be marketed.

6.6 Special precautions for disposal and other handling

No special requirements.

7. PRODUCT REGISTRATION HOLDER

Sandoz Products Malaysia Sdn. Bhd.
Unit 1202, Level 12, Uptown 1,
No. 1, Jalan SS 21/58, Damansara Uptown,
47400 Petaling Jaya, Selangor, Malaysia.

8. DATE OF REVISION OF THE TEXT

Apr 2025